Patient Record — ID 1764

Age	28	**Sex**	F
Race/Ethnicity	Non-Hispanic White	**Insurance**	Blue Cross Blue Shield PPO
Total Encounters	8	**Generation Seed**	3725

Primary Diagnoses

• Breech presentation at term
• Asymptomatic Actinomyces colonization of genital tract
• Malignant melanoma
• Anencephaly
• High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy
• Group B Streptococcus carrier state complicating pregnancy
• Pregnancy (20 weeks gestation)
• False labor (Braxton Hicks contractions)

Comorbidities

• Term pregnancy
• Pregnancy (20 weeks gestation)
• Pregnancy, 38 weeks gestation (third trimester)
• Nulliparity

Patient Profile

Sex	F
Age at First Encounter	28
Race/Ethnicity	Non-Hispanic White
Insurance	Blue Cross Blue Shield PPO
Occupation	Elementary school teacher
Smoking Status	Never smoker
Alcohol Use	None

Allergies:

• No known drug allergies

Chronic Conditions:

• Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
• History of malignant melanoma (back, status post wide local excision)
• History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Home Medications:

• Carbamazepine - 200 mg PO twice daily
• Folic acid - 4 mg PO daily

Surgical History:

• Wide local excision with 1cm margins, right upper back (melanoma)
• Cervical colposcopy (unsatisfactory visualization)
• Sentinel lymph node biopsy, negative for malignancy

Family History:

• Maternal aunt: Malignant melanoma (age 45)
• Paternal grandmother: Epilepsy
• Mother: Hypothyroidism
• Father: Type 2 diabetes mellitus

Encounters (8)

Encounter 1 — 2020-01-15 — Dermatology

Encounter ID	7343	**Type**	outpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Dermatology	**Gen. Method**	template

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Concerns about a changing mole on her back that her husband recently noticed has become darker and slightly larger over the past two months.

HPI

She reports that the mole has become darker and slightly larger over the past two months. She denies any pain, itching, or bleeding from the site.

PMH (modified)

Active Problem List:
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY

She has no family history of skin cancer.

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

PHYSICAL_EXAM

On examination, the mole measures 8 mm in diameter, has irregular borders, and exhibits variegated colors including brown, black, and a hint of blue. There is no evidence of ulceration or bleeding. The remainder of her skin examination is normal.

ASSESSMENT

Given the changes in the mole during pregnancy,

Encounter 2 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	7345	**Type**	outpatient
Attending	Dr. Maria Rodriguez	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	template

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Routine gynecologic examination with abnormal cervical cancer screening results requiring colposcopic evaluation.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

Encounter 3 — 2021-07-15 — Obstetrics & Gynecology

Encounter ID	7342	**Type**	outpatient
Attending	Dr. Jennifer Park	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Routine annual well-woman examination with no active complaints.

HPI (modified)

The patient presents for routine annual well-woman examination with a history of generalized seizure disorder, malignant melanoma status post wide local excision, and high-grade squamous intraepithelial lesion of the cervix following prior dermatologic evaluation for a changing back mole and gynecologic assessment for abnormal cervical cancer screening. She has no complaints.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

She has never been pregnant and has regular menstrual cycles. Her last Pap smear was two years ago and was normal.

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY

She is in a monogamous relationship

VITALS

On examination, her vital signs are within normal limits

PHYSICAL_EXAM

Her pelvic exam is unremarkable with no cervical motion tenderness or adnexal masses

PATHOLOGY

A Pap smear is performed, and the results show the presence of Actinomyces-like organisms.

ASSESSMENT

The patient remains asymptomatic.

Encounter 4 — 2022-05-15 — Obstetrics & Gynecology

Encounter ID	7347	**Type**	outpatient
Attending	Dr. Emily Johnson	**Order**	3
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

First prenatal visit at approximately 10 weeks gestation with mild nausea and fatigue.

HPI (modified)

This 30-year-old patient with epilepsy, history of malignant melanoma, and high-grade cervical dysplasia presents for her first prenatal visit following prior gynecologic care. She reports feeling well with mild nausea and fatigue.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her immunizations are up to date.

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES

She has no known drug allergies

FAMILY_HISTORY

Her family history is non-contributory.

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITALS

On physical examination, her blood pressure is 120/78 mmHg, heart rate is 72 beats per minute, and she is afebrile.

PHYSICAL_EXAM

Her abdomen is soft, non-tender, with no palpable masses. Pelvic examination is normal for gestational age.

LABS

Laboratory results are as follows:

- Blood type: A negative
- Antibody screen: Negative
- Hemoglobin: 12.5 g/dL
- White blood cell count: 7,000/uL
- Platelets: 250,000/uL

ASSESSMENT

Given the patient's blood type and antibody screen,

Encounter 5 — 2022-08-15 — Obstetrics & Gynecology

Encounter ID	7344	**Type**	outpatient
Attending	Dr. Emily Johnson	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Routine prenatal visit at 20 weeks gestation for second-trimester screening.

HPI (modified)

She has melanoma, HSIL, epilepsy. She returns for 20-week visit reporting feeling well with uneventful pregnancy thus far.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her medical history is significant for epilepsy, for which she takes carbamazepine.

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

She has been taking prenatal vitamins since she found out she was pregnant at 8 weeks gestation.

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY

She does not smoke or drink alcohol.

VITALS

On examination, her blood pressure is 120/78 mmHg, heart rate is 72 beats per minute,

PHYSICAL_EXAM

Fetal heart tones are normal.

IMAGING

A routine fetal ultrasound is performed, which reveals an absent cerebrum, a calvarial defect, and an abnormal cerebellum. There is also evidence of polyhydramnios.

Encounter 6 — 2022-12-15 — Emergency Medicine

Encounter ID	7348	**Type**	ed
Attending	Dr. David Kim	**Order**	5
Department	Emergency Medicine	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Mild, irregular contractions every 10 to 15 minutes for the past 3 hours without vaginal bleeding or fluid leakage.

HPI (modified)

This 30-year-old patient with new-onset epilepsy and history of malignant melanoma and cervical HSIL presents at 36 weeks gestation. She describes the contractions as mild and irregular, occurring approximately every 10 to 15 minutes, without any increase in intensity. She denies any vaginal bleeding, leakage of fluid, or decrease in fetal movement.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

Her pregnancy has been uncomplicated, and prenatal care has been adequate.

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITALS

On physical examination, her vital signs are stable,

PHYSICAL_EXAM

The fetal heart rate tracing is reassuring. A speculum examination reveals no evidence of ruptured membranes, and the cervix is closed, long, and posterior. No cervical change is noted compared to her last prenatal visit a week ago.

LABS

Laboratory Values:
- Hemoglobin: 12.2 g/dL
- White blood cell count: 9,000/mm^3
- Platelets: 250,000/mm^3
- Urinalysis: No proteinuria or glucosuria

Encounter 7 — 2023-02-15 — Obstetrics & Gynecology

Encounter ID	7341	**Type**	outpatient
Attending	Dr. Lisa Thompson	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Routine prenatal visit at term with discussion of delivery plans given suspected fetal malpresentation.

HPI (modified)

This 31-year-old with malignant melanoma, cervical HSIL, and epilepsy returns for prenatal care at term. She reports feeling well with good fetal movement. Her pregnancy has been uncomplicated, and she has received regular prenatal care. The patient expresses a desire for a vaginal delivery if possible.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- False labor (Braxton Hicks contractions) (diagnosed 2022-12-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITALS

On examination, her blood pressure is 120/78 mmHg, heart rate is 72 bpm, and fetal heart rate is 140 bpm.

PHYSICAL_EXAM

The obstetric examination reveals a fundal height consistent with gestational age, but the fetal position is noted to be breech on Leopold maneuvers.

IMAGING

An ultrasound confirms a frank breech presentation with no other abnormalities. Amniotic fluid index is normal, and there are no signs of placental insufficiency.

Encounter 8 — 2023-03-15 — Obstetrics & Gynecology

Encounter ID	7346	**Type**	inpatient
Attending	Dr. Robert Garcia	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Structured EHR Sections:

CHIEF_COMPLAINT (modified)

Regular contractions every 5 minutes for 2 hours with spontaneous rupture of membranes 30 minutes prior.

HPI (modified)

The 31-year-old with melanoma, HSIL, and epilepsy since 12 weeks gestation returns following breech presentation. She reports her water broke 30 minutes ago. Her prenatal course was complicated by asymptomatic bacteriuria at 16 weeks caused by GBS and treated with amoxicillin.

PMH (modified)

Active Problem List:
- Malignant melanoma (diagnosed 2020-01-15)
- High-grade squamous intraepithelial lesion of cervix with unsatisfactory colposcopy (diagnosed 2020-03-15)
- Asymptomatic Actinomyces colonization of genital tract (diagnosed 2021-07-15)
- Pregnancy (20 weeks gestation) (diagnosed 2022-05-15)
- Anencephaly (diagnosed 2022-08-15)
- False labor (Braxton Hicks contractions) (diagnosed 2022-12-15)
- Breech presentation at term (diagnosed 2023-02-15)
- Epilepsy (generalized seizure disorder, new onset at 12 weeks gestation)
- History of malignant melanoma (back, status post wide local excision)
- History of high-grade squamous intraepithelial lesion (HSIL/CIN 2-3) of the cervix

PSH (modified)

- Wide local excision with 1cm margins, right upper back (melanoma)
- Cervical colposcopy (unsatisfactory visualization)
- Sentinel lymph node biopsy, negative for malignancy

MEDICATIONS (modified)

Home Medications:
- Carbamazepine 200 mg PO twice daily
- Folic acid 4 mg PO daily

ALLERGIES (modified)

No known drug allergies

FAMILY_HISTORY (modified)

- Maternal aunt: Malignant melanoma (age 45)
- Paternal grandmother: Epilepsy
- Mother: Hypothyroidism
- Father: Type 2 diabetes mellitus

SOCIAL_HISTORY (modified)

Smoking: Never smoker. Alcohol: None. Occupation: Elementary school teacher.

VITALS

Her temperature is 37.2°C (99.0°F), blood pressure is 120/78 mmHg, pulse is 88/min, and respiratory rate is 16/min.

PHYSICAL_EXAM

Physical examination reveals a gravid abdomen with a fundal height appropriate for gestational age. Fetal heart tones are regular at 140 beats/min. Pelvic examination shows that the cervix is dilated to 4 cm, effaced 80%, and the amniotic membranes are ruptured.

ASSESSMENT

Given her medical history,